Riverside General Hospital - Cardiology Department

CARDIOLOGY CONSULTATION REPORT

Patient:	John A. Doe	MRN:	00482917
DOB:	03/14/1958 (68 y)	Sex:	Male
Date of Study:	10/24/2026	Referring MD:	Dr. Sarah Mitchell
Cardiologist:	Dr. Priya Nair, MD, FACC	Location:	Inpatient, Telemetry Unit

Reason for Consultation

Evaluation of exertional chest pressure in a 68-year-old man with hypertension, Type 2 diabetes, and hyperlipidemia. Rule out obstructive coronary artery disease.

Cardiac History

No prior history of myocardial infarction, coronary revascularization, arrhythmia, or heart failure. No prior cardiac testing on file. Risk factors: hypertension, diabetes, dyslipidemia, former tobacco use, family history of coronary disease (father, age 66).

Current Cardiac-Relevant Medications

Lisinopril, Atorvastatin 40 mg at bedtime, Metformin 500 mg twice daily.

Physical Examination

BP 152/90 mmHg, HR 76 bpm regular, RR 16, SpO2 98%. JVP not elevated. Regular rate and rhythm, normal S1/S2, soft grade 1/6 systolic ejection murmur at the left upper sternal border. Lungs clear. No peripheral edema.

Electrocardiogram

Normal sinus rhythm, rate 78 bpm. PR 164 ms, QRS 92 ms, QTc 428 ms. Nonspecific T-wave flattening in leads V4-V6. No pathologic Q waves. No ST-segment deviation.

Transthoracic Echocardiogram

Parameter	Measurement	Interpretation
LVEF (biplane)	58%	Normal (reference 52-72%)
LV septal wall thickness	12 mm	Mildly increased
LV end-diastolic diameter	48 mm	Normal
Left atrial volume index	30 mL/m2	Upper-normal
E/e' ratio	10	Normal filling pressure
Aortic valve	Mild sclerosis, no stenosis	Peak velocity 1.8 m/s
Mitral valve	Trace regurgitation	Physiologic
Pericardium	No effusion	Normal

Conclusion: Preserved systolic function with mild concentric LV hypertrophy, consistent with chronic hypertension. No regional wall motion abnormalities.

Pharmacologic Stress Test (Regadenoson SPECT)

Regadenoson 0.4 mg IV was administered. The patient reported mild flushing and brief dyspnea, which resolved within 3 minutes. No chest pain during the study. No arrhythmias. No ischemic ST-segment changes.

Finding	Result
Baseline BP / HR	148/88 mmHg / 74 bpm
Peak HR	94 bpm
Perfusion imaging	Homogeneous tracer uptake, no reversible or fixed defects
Gated LVEF (post-stress)	60%
Transient ischemic dilation	Absent
Coronary artery calcium (CT attenuation)	Mild, left anterior descending territory

Impression: Negative for inducible myocardial ischemia. Low-risk study.

Assessment

1. Chest pain: unlikely to represent obstructive coronary disease based on normal troponins, negative stress imaging, and preserved ejection fraction.
2. Hypertensive heart disease with mild LVH; blood pressure above goal on admission.
3. Hyperlipidemia: LDL 138 mg/dL, above goal for a patient with diabetes. Mild coronary calcification noted.
4. No history of myocardial infarction; no evidence of prior infarct on imaging.

Recommendations

1. Continue Lisinopril 10 mg daily (increased from 5 mg); target BP below 130/80 mmHg.
2. Continue Atorvastatin 40 mg daily at bedtime. Recheck lipid panel in 8-12 weeks; consider increasing to 80 mg or adding ezetimibe if LDL remains above 70 mg/dL.
3. Continue Metformin 500 mg twice daily per primary care.
4. Consider low-dose aspirin only after shared decision-making given bleeding risk; not started at this time.
5. Lifestyle: low-sodium diet, regular aerobic exercise, continued tobacco abstinence.
6. Outpatient cardiology follow-up in 4 weeks with repeat blood pressure review. No further ischemic testing is needed unless symptoms recur.

Follow-up Echocardiogram

Repeat echocardiogram in 12-24 months to reassess LV hypertrophy if blood pressure control is suboptimal.

Electronically signed: Priya Nair, MD, FACC - 10/24/2026 11:15

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